TB-12 Abdominal / Pelvic Trauma

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | Reconciled October 17, 2026

Core principle: The abdomen, retroperitoneum, and pelvis can conceal life-threatening hemorrhage before hypotension appears. Control external bleeding, stabilize suspected pelvic fracture, prevent hypothermia, and move rapidly toward definitive hemorrhage control.

Field flow

1. XABCDE Control external hemorrhage, support airway/ventilation, assess perfusion and neurologic status, expose only as needed, and warm.	2. Internal hemorrhage? Shock, pain/tenderness, distention, seatbelt sign, flank bruising, high-risk mechanism, pelvic pain/deformity, or unreliable examination.	3. Open injury? Do not remove an impaled object or reduce evisceration. Stabilize and cover without pressure.	4. Resuscitate en route IV/IO, warming, adult LR 1 L bolus with reassessment, analgesia/antiemetic, and Paramedic TXA for life-threatening hemorrhage.

Provider-level actions

Level	Approved actions
EMT	Perform XABCDE, control external hemorrhage, prevent hypothermia, and assess abdomen, flanks, pelvis, groin, and perineum. Do not repeatedly rock the pelvis. Apply the approved binder over the greater trochanters when indicated. Package for rapid trauma transport.
AEMT	All EMT care plus IV/IO access. For an adult with traumatic shock, give lactated Ringer's 1 L IV/IO, then reassess perfusion, respiratory status, and response. Do not delay transport for access or fluid; avoid unnecessary additional large-volume crystalloid.
Paramedic	All prior care plus advanced airway/ventilation, monitoring, shock resuscitation, Medical Control, and destination decisions. For life-threatening abdominal or pelvic hemorrhage, administer TXA under the approved Claiborne TXA pathway; do not delay transport or definitive hemorrhage control.

High-risk findings

Early trauma-center alert: shock; penetrating torso injury; evisceration or impalement; expanding abdominal, flank, or groin hematoma; pelvic deformity or pain after high-energy trauma; seatbelt sign; guarding or rigidity; hematuria or blood at the meatus; pregnancy; pediatric or older patient; anticoagulant use; or unreliable examination.

Rapid assessment

• Inspect and gently palpate the abdomen and flanks once for pain, guarding, rigidity, distention, bruising, penetrating wounds, evisceration, or exposed organs. Do not repeatedly examine a painful abdomen.

• Inspect pelvis, groin, and perineum for wounds, bruising, deformity, swelling, blood at the urinary meatus, hematuria, rectal or vaginal bleeding, or neurologic deficit while preserving privacy and warmth.

• Trend mental status, pulse quality, skin findings, blood pressure, pulse pressure, respiratory status, and response to resuscitation. A normal initial blood pressure does not exclude compensated hemorrhage.

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PELVIC STABILIZATION, RESUSCITATION, AND TRANSPORT

Pelvic stabilization

• Use significant mechanism plus pelvic pain, deformity, shock, or unreliable examination to identify a suspected pelvic fracture. Do not repeatedly rock or compress the pelvis to test stability.

• Apply the agency-approved commercial pelvic binder centered over the greater trochanters, not the abdomen, waist, or iliac crests. Secure the legs together when appropriate and reassess distal circulation, sensation, and movement.

• Minimize movement. Do not loosen or remove a correctly positioned binder in the field unless a separately approved protocol or Medical Control requires it.

Evisceration and impaled object

Finding	Approved response
Evisceration	Do not replace organs. Cover loosely with sterile saline-moistened dressings and an occlusive layer when available. Avoid direct pressure and protect from heat loss.
Impaled object	Do not remove. Stabilize in place with bulky dressings without pressure on exposed organs. Modify or remove only for an immediate lifesaving intervention or safe transport under rescue/Medical Control coordination.
External bleeding	Use direct pressure, wound packing, hemostatic gauze, or tourniquet for compressible external or junctional hemorrhage when anatomically appropriate. Do not blindly pack the abdominal cavity.

Resuscitation and destination

• Establish IV/IO access only when it will not delay transport. For adult traumatic shock, give lactated Ringer's 1 L IV/IO, then reassess perfusion, respiratory status, and response.

• TXA is a Paramedic intervention for life-threatening abdominal or pelvic hemorrhage under the approved Claiborne TXA pathway. Do not delay transport or definitive hemorrhage control for TXA administration.

• Provide approved analgesia and antiemetic care, keep the patient NPO, prevent hypothermia, reassess frequently, and notify the trauma center early.

• For pregnancy beyond approximately 20 weeks, prioritize maternal resuscitation and use manual left uterine displacement or left lateral tilt when compatible with trauma care. Pediatric hypotension is a late finding; use an appropriately sized pelvic device only when indicated.

Do not delay definitive care: Complete nonessential assessment and procedures during transport. Never remove an impaled object, reduce eviscerated organs, apply direct pressure to exposed viscera, or blindly pack the abdominal cavity.

Documentation and handoff

• Document mechanism, serial findings and vital signs, suspected bleeding source, binder indication/position/time, distal neurovascular findings, open-injury care, fluids, TXA pathway use, medications, response, pregnancy status, consultation, and destination rationale.

• This direct source PDF replaces the prior source and matches Claiborne-approved app content reconciled October 17, 2026.